

Trust Policy		 University Hospitals Plymouth NHS Trust
---------------------	---	--

Essential Adult Inpatient Observations, Reporting and Escalation Policy

Issue Date	Review Date	Version
December 2024	December 2029	4

Purpose

This policy sets out the criteria for performing consistent and safe essential adult inpatient observations. In addition, it describes the action that must be taken in line with the Adult Inpatient Observation Chart and Escalation Procedure.

Who should read this document?

All clinical staff involved in measuring, recording, escalating, and assessing physiological observations (vital signs) for adult inpatients at UHP.

Key Messages

This policy refers to the essential adult inpatient observations, which must be undertaken as a minimum and must be recorded on the Trust's NEWS2 chart which will then guide the practitioner in escalating a deteriorating patient appropriately.

Core accountabilities

Owner	Lead Nurse Acute Care Team
Consultation	Matrons and key Trust personnel
Ratification	Nursing and Midwifery Operational Committee (NMOC) –
Dissemination	Heads of Nursing, Matrons and Senior Nurses
Compliance	Nursing and Midwifery Operational Committee (NMOC)

Links to other policies and procedures

Consent to Examination or Treatment Policy TRW.HGV.POL.216.8
Hand Hygiene Guidelines CLI.INF.GUI.55.9.
Mental Capacity 2005.

Version History

1	August 2016	Final Document
2	August 2021	Reviewed and updated
3	June 2023	Reviewed and updated
4	December 2024	Reviewed and updated

Equality, diversity, and inclusion is at the heart of the NHS strategy and for UHP this means ensuring we create an equitable organisation for colleagues, patients, and visitors. Our inclusion agenda recognises that everyone matters, as well as understanding that there are times when people, particularly those from protected groups (defined by the Equality Act 2010 - age; disability; gender reassignment; marriage and civil partnership;

pregnancy and maternity; race; religion or belief; sex; sexual orientation), may face unfairness and discrimination.

We have a moral responsibility to work in a way that creates fairness as well as a legal and public sector equality duty which we take very seriously. We will always aim to create policies and practices that eliminate workplace discrimination and health inequalities, as well as strengthening our relationships within the community and with our colleagues who may identify with one or more protected groups.

A diverse workforce improves the experience of our patients and creates an environment where everyone belongs and feels included. UHP recognise that we all play a part in providing exceptional care to patients as well as showing kindness and compassion to patients and colleagues.

An electronic version of this document is available in the Document Library. Larger text, Braille and Audio versions can be made available upon request.

Contents

Section	Description	Page
1	Introduction	
2	Purpose, including legal or regulatory background	
3	Definitions	
4	Duties	
5	Main Body of Policy (can be as many sections as required)	
6	Overall Responsibility for the Document	
7	Consultation and Ratification	
8	Dissemination and Implementation	
9	Monitoring Compliance and Effectiveness	
10	References and Associated Documentation	
Appendix 1	Dissemination Plan and Review Checklist	
Appendix 2	Equality Impact Assessment	

1 Introduction

Physiological observations are fundamental to ensuring that patients in hospital are safe and that healthcare professionals are aware of the health status of patients (NICE, 2007;NPSA 2007).

All patients in acute hospital settings must have regular comprehensive physiological observations as a minimum standard. The identification of abnormal observations or those causing concern must be followed by a prompt and appropriate level of escalation to the medical team and/or Acute Care Team (ACT). This will ensure that any deterioration in a patient's condition will be recognized early, treated promptly by qualified and suitably trained staff.

The essential observations to be recorded are:

- Respiratory rate
- Oxygen saturation
- Blood Pressure
- Heart Rate
- Temperature

2 Purpose

This policy sets out the criteria for performing essential adult inpatient observations to a consistent and safe standard. In addition, it describes the action that must be taken in line with the Adult Inpatient Observation Chart and Escalation Procedure.

The policy does not cover advanced monitoring of cardiopulmonary observations such as continuous electrocardiogram (ECG) monitoring, invasive arterial blood pressure measurement, or central venous pressure (CVP) monitoring.

This policy applies to all adult patients admitted to the Hospital as inpatients, including the Emergency Department but excluding Critical Care areas. Please refer to separate guidance for these patient groups. Staff in these areas must adhere to specialist practice as instructed through their local induction and development programs.

3 Definitions

Acute Care Team- ACT. Electrocardiogram- ECG, Central Venous Pressure- CVP, Early Warning Score- EWS. Glasgow Coma Score- GCS.

4 Duties

Plymouth Hospitals NHS Trust is an established teaching Trust and has a range of undergraduate students in medicine, nursing, midwifery and allied health professions. In addition, the Trust has an active program of support worker development. Where it is appropriate to their role, students/trainees should be able to observe and practice as part of the learning process under the direct supervision of the registered practitioner or competent healthcare professional.

Plymouth Hospitals NHS Trust has an established Early Warning Scoring (NEWS2) chart.

The Trust has a duty to patients to ensure that there is adequately trained staff to ensure that the procedure of taking and recording vital signs is undertaken completely every time and they are done at a frequency that is determined by the patient's presenting condition. The trust also has a duty to ensure that staff know the correct reporting; and escalation procedures in accordance with this policy 24 hours a day.

Service Line Management Team

It is the responsibility of the Service Line Management team to ensure that staff are released for training and aware of the changes highlighted in the policy.

Matrons

It is the responsibility of the Matrons to ensure that their staff have attended and are up to date with relevant training for their role; and accurate records are kept and recorded on Oracle Learning Management (OLM) system.

Ward Manager

It is the responsibility of the Ward Manager to follow up reports regarding staff that Did Not Attend relevant training and to take appropriate action to ensure staff meet the trust's mandatory requirements.

Medical Educational Supervisors

It is the role of the Medical Educational Supervisors to ensure that their medical trainees have completed the relevant EWS training/ awareness programs.

Learning and Development Team

Learning and Development can be commissioned to support the delivery of education / training related to essential adult inpatient observations.

It is the responsibility of the Learning and Development Department to ensure that all relevant training registers are uploaded onto the OLM within 3 weeks of the course / learning being undertaken.

All Clinical Staff

Individual staff members undertaking clinical observations are responsible for ensuring they are suitably trained and competent to undertake the skill.

5 Main Body of Policy

This policy refers to the following essential adult inpatient observations, which must be undertaken as a minimum. These observations must be recorded on the Trust's NEWS2 chart which will indicate the appropriate level of escalation based on the degree of abnormality of physiological parameters. The following physiological parameters are the essential observations, and these will be recorded in the following priority order at each set of observations taken:

- Respiratory Rate
- Oxygen Saturations
- Blood Pressure
- Pulse
- Neuro Response – ACVPU / Glasgow Coma Score (GCS)
- Temperature

In addition to this, patient's pain score must be assessed and recorded at each set of observations. The pain score is measured out of 3. 0= None, 1+ Mild, 2= Moderate, 3= severe. In specific clinical circumstances, additional physiological monitoring should be considered; this will depend on the clinical area and the condition of the patient.

Examples are:

Blood loss via drains or wounds

Peripheral sensation and/or movement

Capillary refill – pressure points

Urine output.

Consent

Every time a set of observations is to be taken, the practitioner must gain consent from the patient. Where the patient lacks capacity to consent, the practitioner can undertake observations if this is in

the patient's best interests in line with Best Interest Policy/Procedure this must be documented in the patients notes.

Equipment

All clinical areas must have:

- **Manual blood pressure sphygmomanometers and stethoscopes.**
- Tympanic thermometers
- Pulse oximeters

All equipment must be checked prior to each use or as a minimum daily if not in use and be in clean working order.

Any equipment that is damaged or not working must be sent to the medical equipment management team for repair.

Regular Observations

- All patients must have a set of manual essential observations recorded on admission to hospital. Manual observations must be identified as 'M' on the observation chart.
- Each patient must have at least one set of manual observations each day
- Any patient with an irregular and/or fast heart rate **must always** have their blood pressure and heart rate recorded **manually**.
- Any observation that lies on a scoring border must be counted as the higher score.
- Any observation that is un-recordable must be scored as RED and escalated immediately.
- All observations must be recorded with **solid dots and then join the dots with straight solid lines**)

Oxygen Saturations

Target oxygen saturations need to be documented on the NEWs 2 chart and EPMA by the admitting medical staff. If oxygen is required, this must be prescribed prior to administration. ONLY in an emergency can oxygen be prescribed after administration starts. Nursing staff are required to check the oxygen flow rate and device in use at every drug round and document this in the appropriate section in oxygen section of the inpatient prescription chart. If a Venturi valve is used the % of oxygen on the valve should be charted.

Frequency of Observations

The required frequency of essential physiological observations must be recorded on the observation chart, by the Registered Nurse or Doctor. Every patient must have all observations recorded every time at the prescribed frequency unless they have a clearly documented plan that specifies which physiological observations should be recorded and how often.

All inpatients will have their essential physiological observations recorded **every 12 hours** as a minimum. This frequency may be reduced so long as this is documented in the patient's record by a senior member of the medical team.

The frequency of essential physiological observations will be guided by the current level of escalation. Where a patient is displaying signs of deterioration, the frequency of observations will be increased as per instructions on the observation chart.

On admission the admitting medical team must document the frequency of observations required for each individual patient in the patient's clinical record. This frequency must be reviewed as part of the patient's medical review.

The medical team must consider revising the trigger point depending on the patient's condition. The revised trigger point can be adjusted, therefore increasing or decreasing the threshold for triggering a medical review. The trigger point can be adjusted by a Foundation Year doctor, however this must first be discussed with a Specialty Registrar (ST3+ or above) or Consultant and documented on the observation chart and within the patient notes. The revised trigger must be reviewed during the next Consultant review of that patient.

For emergency admissions, acutely unwell patients and patients stepping down from designated critical care areas, observations must be taken at least every 4 hours until a senior medical review has occurred. This should continue until the patient is seen by the medical team and agreed frequency of observations has been documented.

This does not apply to patients on the End-of-Life care where a decision has been made to stop observations.

For patients where the minimum observation frequency has been significantly reduced, a set of observations must be performed within 24 hours of discharge, to ensure that there have been no significant changes to the patient's condition.

Post general/spinal anesthesia observations

For inpatients who have undergone general or spinal anesthesia, observations will be performed at the following minimum frequencies post-procedure, unless a decision is made at a senior medical level to reduce this frequency.

Specific post-operative pathways may also necessitate observations at more or less frequent intervals. Please refer to locally agreed guidelines/pathways for more information.

- Every 30 minutes for two hours
- Hourly for four hours
- Two hourly for six hours
- Four hourly for twelve hours

For the purposes of this policy, the patient's arrival on the ward indicates the start of the post-operative period.

This will make a total of 24 hours monitoring post procedure. An increased frequency of observations may be necessary depending on triggers/concerns raised during the post-operative period.

Observations **MUST** be recorded overnight, where there is a clinical indication to do so. There is no qualifiable evidence to omit post-operative observations because a patient is asleep.

Documentation

All documentation must be clear and legible, using consistent methods of charting as illustrated in Appendix.

All documentation must be completed in black ink or entered on to the appropriate digital observation chart.

Manual observations must be identified on the observation charts with 'M'.

The time recorded on the observation chart must be the exact time the observations were recorded.

It is acceptable to identify lying and standing blood pressure with 'Ly' or 'St', but both recordings must be with black ink.

It is good practice to identify which arm has been used for blood pressure measurement (Left or Right). Or when indicated, which leg.

All observations causing concern and the action taken must be documented in the patient record and also on the back of the observation chart if a trigger is identified.

Reporting and Escalation

Unregistered practitioners must report any change in the patient's observations to the registered practitioner who delegated the task to them. This must be done verbally, and the action taken recorded on the back of the observation chart

All action taken by Registered and Non-Registered staff in response to the observations must be recorded on the reverse of the observation chart.

The Situation, Background, Assessment, Recommendation (SBAR) format must be used to formally escalate a patient who is presenting with deterioration in their physiological status or is a cause for concern, in every event a full set of observations must be available when reporting deterioration of a patient's condition.

All medical staff of grade FY2 and above and Acute Care Team clinical staff must assess, manage and escalate patients using an ABCDE approach. Assessment should include consideration and documentation of:

- The cause of deterioration
- The management plan with guidance to nursing staff in relation to fluid therapy and Oxygen.
- The timing of the next medical review
- The frequency of observations
- The ceiling of treatment including Cardiopulmonary Resuscitation decisions (discussion with SpR or above)
- Adjusting the trigger if appropriate (discussion with ST3+ or above).
- Removing the patient from the NEWS 2 trigger system if appropriate (discussion with ST3+ or above)
- Escalation to Acute Care Team and senior medical teams if patient continues to deteriorate or escalation to Acute Care Team if NEWS of 5 or above.
- If nursing staff have any concerns about the escalation or the timeliness of review then they must contact Acute Care Team

Training

Clinical observations and SBAR training is essential for all clinical staff who undertake essential observations.

Staff can access classroom, ward based or e-learning training materials to undertake this.

All training including essential observations must be based on this policy and training staff will refer to it for guidance.

Ensuring that staff are competent in performing essential observations must be included in all ward/department induction programs.

Training records for all staff must be kept by the ward/department manager and entered on to OLM for compliance, governance and audit purposes.

Infection Control

Clinical practice covered within this policy must adhere to the standards set out in the Trust's Infection Prevention and Control policies and procedures.

Maternal Essential Observations

The Maternal Obstetric Early Warning (MOEW) Chart utilizes a different trigger system for summoning medical assistance based on the Confidential Enquiry into Maternal and Child Health (2007) recommendations.

Guidance and instructions for using this trigger system are printed on the back of each MOEW chart and within Maternity digital system.

For further information and a copy of the Maternal Obstetric Early Warning Chart relating to this please refer to the following Maternity Guidelines. These can be found on the Maternal Intranet page

- Enhanced Observations in Maternity
- Maternity Services Training Needs Analysis
- Maternity Patients Attending Emergency department.

Training in the use of the Maternal Obstetric Early Warning chart will be included within the Maternity mandatory training programs as identified in the Maternity Training Needs Analysis.

Any Obstetric patient on a non-maternity ward must be referred to the Central Delivery Suite (CDS) Coordinator on bleep 1042 by the ward staff. Either a Midwife or Obstetric medical staff will then review the patient as necessary, recording any maternal observations on a Maternal Obstetric Early warning chart, which is to be stored in the patient's notes. All observations undertaken by nursing staff will continue to be recorded on the National NEWs 2 Chart. (Obstetric SHO carries Bleep 0464 and Obstetric Registrar has bleep 0311.)

ICU, CDU, and Theatres

All Patients who are going to be transferred to the wards must already be on the NEWs 2 Chart.

Any patient who triggers a response must first be medically assessed prior to transfer.

ICU

All patients who are ready to return to the ward must have observations recorded on the NEWs 2 Chart. Escalation should be made to the ICU medical staff in the event of any patient triggering a response.

Any trigger and subsequent intervention must be clearly documented and the receiving ward informed.

Emergency Department

ED staff will complete a NEWS 2 chart at triage and at the point of patient handover to the ward. Observations on a NEWS 2 chart will be recorded whilst the patient is in the department and the local escalation process will be followed in the event of deterioration.

The Acute Care Team must be notified of any patient transferring to an inpatient ward from the ED's Resus area.

Theatres

All Patients must have regular observations recorded on the NEWS 2 Chart and score recorded prior to discharge back to a clinical area post operatively. Escalation must be made to the Theatre doctors in the event of any patient triggering a response prior to discharge.

Maintenance of Equipment

Where applicable calibration and accuracy of equipment should be regularly checked based on the manufacturer's recommendation or annually as per Trust policy.

6 Overall Responsibility for the Document

This policy will be overseen, monitored and updated via the Nursing & Midwifery Operational Committee (NMOC), chaired by the Chief Nurse.

7 Consultation and Ratification

The design and process of review and revision of this policy will comply with The Development and Management of Formal Documents.

The review period for this document is set as default of five years from the date it was last ratified, or earlier if developments within or external to the Trust indicate the need for a significant revision to the procedures described.

This document will be reviewed by the Nursing & Midwifery Operational Committee (NMOC) and ratified by the Director of Nursing.

Non-significant amendments to this document may be made, under delegated authority from the Director, by the nominated owner. These must be ratified by the Director.

Significant reviews and revisions to this document will include a consultation with named groups, or grades across the Trust. For non-significant amendments, informal consultation will be restricted to named groups, or grades who are directly affected by the proposed changes.

8 Dissemination and Implementation

Following approval and ratification, this policy will be published in the Trust's formal documents library and all staff will be notified through the Trust's normal notification process.

Document control arrangements will be in accordance with The Development and Management of Formal Documents.

The document owner will be responsible for agreeing the training requirements associated with the newly ratified document with the named Director and for working with the Trust's training function, if required, to arrange for the required training to be delivered.

9 Monitoring Compliance and Effectiveness

Auditing of compliance with this policy and to ensure that essential observations are undertaken as per the policy will take place in all clinical areas (where the chart is used) by the ward staff using the agreed audit collection tool on Meridian. Overarching audit data arising from this audit will be reported to the Nursing and Midwifery Operational Committee on a 6-monthly basis for assurance.

Compliance will also be reported to Director of Nursing, Heads of Nursing and should be reviewed regularly at ward and service line level. Action plans for any identified deficiencies will be developed and monitored at Service Line level, reported through Service Line Governance meetings.

10 References and Associated Documentation

Chatterjee, MT, Moon, JC, Murphy, R, McCrea, D (2005) The "OBS" chart: an evidence based approach to re-design of the patient observation chart in a district general hospital setting, *Postgraduate Medical Journal*, 81, 663-666

Considine, J (2005) The role of nurses in preventing adverse events related to respiratory dysfunction: literature review, *Journal of Advanced Nursing*, 49(6), 624-633

Confidential Enquiry into Maternal and Child Health (2007) Saving Mother's Lives: Reviewing maternal deaths to make motherhood safer www.cemach.org.uk

Greaves, I, Porter, K (eds) (1999) *Pre Hospital Medicine: The principles and practice of immediate care*, London, Arnold

Institute for Healthcare Improvement, Early Warning Systems: Scorecards That Save Lives, www.ihl.org (accessed 2007)

Institute for Healthcare Improvement, Guidelines for Communicating with Physicians Using the SBAR Process, www.ihl.org (accessed 2007)

Joanna Briggs Institute for Evidence Based Nursing and Midwifery, Best Practice: Vital Signs, www.joannabriggs.edu.au/best_practice/bp8.php, (accessed 2006)

Mackay CA, Burke DP, Burke, JA, Porter, KM, Bowden, D, Gorman D (2000) Association between the assessment of conscious level using the AVPU system and the Glasgow coma scale, *Pre-Hospital Immediate Care*, 4, 17-19

Mallett, J, Dougherty, L (2004) *Observations*, The Royal Marsden Hospital Manual of Clinical Nursing Procedures (6th Edition), Blackwell Science

Medicines and Healthcare products Regulatory Agency (2005) Report of the Independent Advisory Group on Blood pressure Monitoring in Clinical Practice, www.mhra.gov.uk (accessed 2007)

Moulton C, Yates, D (1999 2nd Edition), *The Principles of Emergency Medicine* (Chapter 1), *Lecture Notes on Emergency Medicine*, Blackwell Science

National Confidential Enquiry into Patient Outcome and Death (2005) An Acute Problem?: A report of the National Confidential Enquiry into Patient Outcome and Death, www.ncepod.org.uk (accessed 2006)

National Institute for Health and Clinical Excellence (2007) Acutely ill patients in hospital: Clinical guideline 50, www.nice.org.uk

National Patient Safety Agency (2007) Recognizing and responding appropriately to early signs of deterioration in hospital patients, www.npsa.nhs.uk/corporate/nNEWS/deterioration-in-hospitalised-patients

National Patient Safety Agency (2007) Safer care for the acutely ill patient: learning from serious incidents, www.npsa.nhs.uk/corporate/nNEWS/deterioration-in-hospitalised-patients

Stanford University Medical Centre, Cardiovascular Diseases: Vital Signs, www.stanfordhospital.com/healthLib/atoz/cardiac/vital.html (accessed 2006)

Date Finalized	July 2021		
Previous Documents			
Action to retrieve old copies	N/A		
Dissemination Plan			
Recipient(s)	When	How	Responsibility
All Trust staff	June 2023	Information Governance StaffHub Page	Information Governance Team
Ward Nursing Staff	June 2023	Team Review	Matrons & Senior Sisters

Review Checklist		
Title	Is the title clear and unambiguous?	Yes
	Is it clear whether the document is a policy, procedure, protocol, framework, APN or SOP?	Yes
	Does the style & format comply?	Yes
Rationale	Are reasons for development of the document stated?	Yes
Development Process	Is the method described in brief?	Yes
	Are people involved in the development identified?	Yes
	Has a reasonable attempt has been made to ensure relevant expertise has been used?	Yes
	Is there evidence of consultation with stakeholders and users?	Yes
Content	Is the objective of the document clear?	Yes
	Is the target population clear and unambiguous?	Yes
	Are the intended outcomes described?	Yes
	Are the statements clear and unambiguous?	Yes
Evidence Base	Is the type of evidence to support the document identified explicitly?	Yes
	Are key references cited and in full?	Yes
	Are supporting documents referenced?	Yes
Approval	Does the document identify which committee/group will review it?	Yes
	If appropriate have the joint Human Resources/staff side committee (or equivalent) approved the document?	Yes
	Does the document identify which Executive Director will ratify it?	Yes
Dissemination & Implementation	Is there an outline/plan to identify how this will be done?	Yes
	Does the plan include the necessary training/support to ensure compliance?	Yes

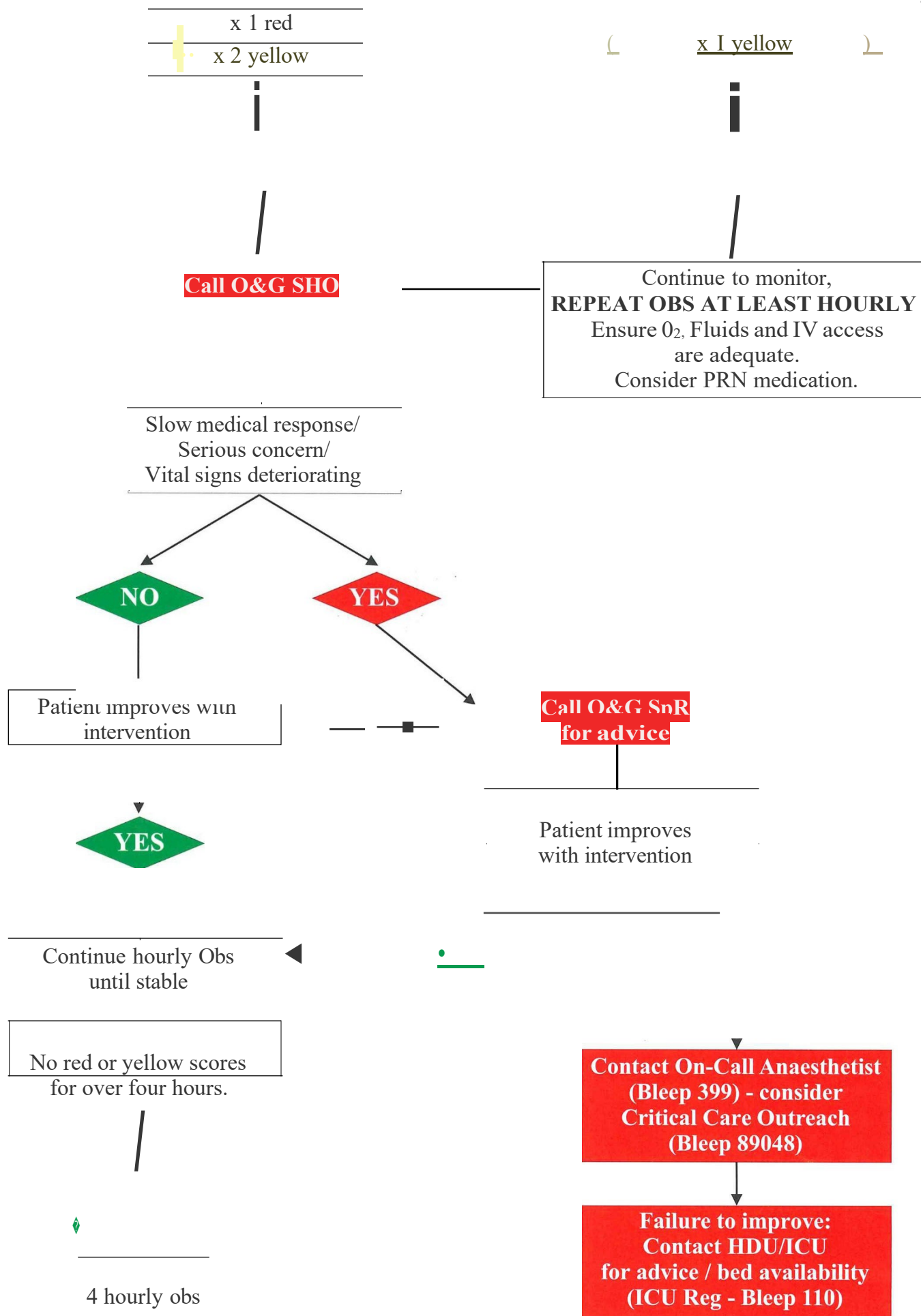
Document Control	Does the document identify where it will be held?	Yes
	Have archiving arrangements for superseded documents been addressed?	Yes
Monitoring Compliance & Effectiveness	Are there measurable standards or KPIs to support the monitoring of compliance with and effectiveness of the document?	Yes
	Is there a plan to review or audit compliance with the document?	Yes
Review Date	Is the review date identified?	Yes
	Is the frequency of review identified? If so is it acceptable?	Yes
Overall Responsibility	Is it clear who will be responsible for coordinating the dissemination, implementation and review of the document?	Yes

Core Information	
Date	13/06/2023
Title	Essential Adult Inpatient Observations, Reporting and Escalation Policy
What are the aims, objectives & projected outcomes?	This policy sets out the criteria for performing consistent and safe essential adult inpatient observations. In addition it describes the action that must be taken in line with the Adult Inpatient Observation Chart and Escalation Procedure.
Scope of the assessment	
The assessment covers all protected characteristics The EIA was produced by the Trust's Equality & Diversity Lead Incidents are monitored via Datix and reported as necessary	
Collecting data	
Race	There is no evidence to suggest there is disproportionate impact on race regarding this policy. Consideration will be made for patients with English as a second language to ensure they are able to provide consent to the procedure Data collected from Datix incident reporting will ensure this is monitored.
Religion	There is no evidence to suggest there is a disproportionate impact on religion or belief and non-belief regarding this policy. Consideration will be made for patients who have specific religious requirements. This will be monitored through complaints and incidents. Data collected from Datix incident reporting will ensure this is monitored.
Disability	There is no evidence to suggest there is a disproportionate impact on disability regarding this policy. However, data collected from Datix incident reporting will ensure this is monitored. Consideration will be made for patients who have special requirements. This will be monitored through complaints and incidents.

Sex	There is no evidence to suggest there is a disproportionate impact on sex regarding this policy. However, data collected from Datix incident reporting will ensure this is monitored.
Gender Identity	There is currently no data collected for this area. However, data collected from Datix incident reporting will ensure this is monitored
Sexual Orientation	There is no evidence to suggest there is disproportionate impact on sexual orientation regarding this policy. However, data collected from Datix incident reporting will ensure this is monitored.
Age	There is no evidence to suggest there is a disproportionate impact on age regarding this policy. However, data collected from Datix incident reporting will ensure this is monitored.
Socio-Economic	There is currently no data collected for this area. However, data collected from Datix incident reporting will ensure this is monitored.
Human Rights	Data collected from Datix incident reporting will ensure this is monitored
What are the overall trends/patterns in the above data?	No comparative data has been used to date which means that no trends or patterns have been identified.
Specific issues and data gaps that may need to be addressed through consultation or further research	No gaps have been identified at this stage but this will be monitored via data collected from Datix incident reporting.

Involving and consulting stakeholders				
Internal involvement and consultation	All Senior Nurse via NMOC Equality and Diversity Lead			
External involvement and consultation	N/A			
Impact Assessment				
Overall assessment and analysis of the evidence				
Action Plan				
Action	Owner	Risks	Completion Date	Progress update
Monitor incidents linked to this policy via Datix			ongoing	

Obstetric Early Warning Flowchart



Patient Sticker

See flow chart
overleaf if patient
triggers one red or
two yellow at any
one time.

Obstetric Early Warning Chart

Date:																			
Time:																			
Temperature	38																		
	37.5																		
	37																		
	36																		
	35																		
Resp. Rate (write rate in corresp. box)	>30																		
	21-30																		
	11-20																		
	0-10																		
SpO₂%	95-100%																		
	<95%																		
Heart Rate / Pulse	140																		
	130																		
	120																		
	110																		
	100																		
	90																		
	80																		
	70																		
	60																		
	50																		
	40																		
Systolic Blood Pressure	200																		
	190																		
	180																		
	170																		
	160																		
	150																		
	140																		
	130																		
	120																		
	110																		
	100																		
Diastolic Blood Pressure	130																		
	120																		
	110																		
	100																		
	90																		
	80																		
	70																		
	60																		
	50																		
	40																		
	Neuro Response (/)	Alert																	
Voice																			
Pain																			
Unresponsive																			
Urine Output (ml/hr)	>50ml/2hrs																		
	<50ml/2hrs																		
	<100ml/4hrs																		
	<10ml/hr																		
Total YELLOW																			
Total RED																			
Initials																			

DATE

TIME

NEWS	EYES	Spontaneously	4															
		To speech	3															
		To pain	2															
		None	1															
	ORIENTED	Orientated	5															
		Confused	4															
		Inappropriate words	3															
		Incomprehensible sounds	2															
	OBEY	None	1															
		Obey commands	6															
		Localise pain	5															
		Flexion to pain	4															
		Abnormal flexion	3															
		Abnormal extension	2															
	NONE	None	1															

COMA SCORE

Perform and record observations half-hourly until GCS 15

For NEW head injury and GCS 15, minimum frequency of observations is

-

Half hourly for 2 hours

Then 1 hourly for 4 hours

Then 2 hourly thereafter

Inform responsible clinician for any;?.2 point drop in GCS and recheck in 30 min.

Pupils	Right	Size																
		Reaction																
	Left	Size																
		Reaction																

PUPILS

+ reacts / - no reaction / c eyes closed by swelling**

** if eyes closed by swelling and unable to open eyelids, report to a senior to review and assess urgently

NEUROVASCULAR

COLOUR Pink (pi) / pale (pa)/ cyanotic (cy)

TEMPERATURE Hot (H) / warm (W) / cool (C) / cold (Cid)

MOVEMENT Present(+) / decreased (..J.) / absent(-)

SENSATION Present(+) / decreased (..J.) / absent (tingling, numbness)

PAIN Mild (mi) / moderate (mo) / severe (s)

NEWS	ALIM	Normal Power																
		Mild weakness																
		Severe weakness																
		Spastic flexion																
		Extension																
		No response																
	SBD	Normal Power																
		Mild weakness																
		Severe weakness																
		Spastic flexion																
Extension																		
No response																		
NEWS	ALIM	Colour																
		Temperature																
		Movement																
		Sensation																
		Pain																
		Capillary refill																
		Pulse (distal)																

University Hospitals
Plymouth
NHS Trust

This chart is for use in
ADULTS only.

For children use PEWS,
For pregnancy use MEOWS.

NAME

DOB

Hosp. No.

AMB = pre-hospital obs
(pre-treatment)
age OD

NEWS Key

ADULT OBSERVATION CHART

NEWS TRIGGER THRESHOLDS

NEW Score

0

Total 1-4

3in single parameter

Total 5 URGENT RESPONSE

Total 7 EMERGENCY RESPONSE

FREQUENCY OF MONITORING

Minimum 12 hrly

Minimum 4-6 hrly

Minimum 1 hrly

Minimum 1 hrly

Continuous monitoring

CLINICAL RESPONSE

Continue routine NEWS monitoring

Registered nurse decides whether increased frequency of monitoring and/or escalation of care is required

Inform responsible clinician (e.g. ward doctor)

Consider sepsis

Immediately inform
ED: lead clinician (EPIC)
Wards: senior doctor & ACT

Consider sepsis

Provide clinical care in an environment with monitoring facilities

Immediately inform
ED: lead clinician (EPIC)
Wards: senior doctor & ACT

Consider sepsis

Consider move to resus / high dependency area

For documenting Sp02 scale, a clinician should confirm the target range.

Please cross off the scale NOT in use.

OXYGEN DELIVERY DEVICES

SM = Simple mask
N = nasal cannulae
RM = reservoir mask
V24 = venturi 24%
(change figure as appropriate)
CP = patient on CPAP
BP = patient on BiPAP
TM = tracheostomy mask

ACT= Acute Care Team

When completing the observations, please document **actual values**.

Once observations documented, please calculate the **NEW Score**.

DATE	TIME	TRIGGER/ COMMENT	ACTION taken (tick box)				PRINT NAME
			NIC Informed	Responsible Clinician review requested	Acute care team informed	Lead Clinician informed	

NAME
DOB
Hosp. No.

- 1 Patient looks unwell *OR*
 - NEWS-2 triggered
 - NEWS 3 in single parameter *OR*
 - **total 5** *OR*
 - Lactate raised 2 mmol/L

- 0 Age > 75
- D Impaired immunity
(e.g. diabetes, steroids, **chemotherapy**)
- D Recent trauma/ surgery/ invasive procedure
- D Indwelling lines, IVDU, broken skin

☐ NO

2 D Source unclear D Brain

Respiratory D Device related

D Urine D Surgical/ Abdomen

D Skin/ Joint/ Wound D Other:

Record that screened for sepsis on NEWS-2

3 **ANY 'I' FLAG PRESENT?**

- D** Objective evidence of NEW altered mental state
- D** SBP > 90 mmHg (or drop of > 40 from normal)
- D** HR 130/min
- D** RR 25/min
- D** Needs O₂ to maintain SpO₂ 92%
- D** Non-blanching rash/ mottled /ashen/ cyanotic
- D** Lactate 2 mmol/L
- D** Recent ↓

YES

This is time critical, immediate action is required(< 1hour)

	ANY	FLAG PRESENT?
4 D	Relatives concerned about mental status	
D	Acute deterioration in functional ability	
D	Immunosuppressed	
D	Trauma/ surgery/ procedure in last 8 weeks	
D	RR 21-24/min	
0	SBP 91-100 mmHg	
0	HR 91-130 or NEW dysrhythmia	
0	Temp < 36°C	
D	Clinical signs of wound infection	Yes

Yes

Further investigations and review required

Send bloods
include lactate, FBC, U&E, CRP, LFT, clotting
Time complete: Initials:

1. $\frac{1}{2}$ 2. $\frac{1}{2}$

Ensure urgent senior review

Must review with results within 1 hour

Time complete: Initials:

1. *Journal of the American Medical Association*, 1997; 278: 1039-1044.

Clinician to make antimicrobial prescribing decision
within 3h

Time complete: Initials:

Antibiotics required: YES ☒ NO ☐

Time complete: _____ Initials: _____

ANY **AMBER** FLAG ~~AND~~ **AKI** present OR **lactate** > 2?

ANY **RED** FLAG PRESENT?

STARTSEP ;PATHWAY

A+B Respirations Breaths/min	>							3									
	21-24							2									
	18-20																
	15-17																
	12-14																
	9-11							1									
								3									
A+B SpO ₂ Scale 1 SpO ₂ Scale 2 Use Scale 2 if target range is 88-92%. Only use under direction of a qualified clinician	>98																
	94-95							1									
	92-93							2									
	<91							3									
	>97							3									
	95-96							2									
	93-94							1									
	>93 on air																
	88-92																
	86-87							1									
	84-85							2									
	<83							3									
	A= Air																
	02 L/min							2									
Device																	
>220							3										
201-219																	
181-200																	
161-180																	
141-160																	
121-140																	
111-120																	
101-110							1										
91-100							2										
81-90																	
71-80																	
61-70							3										
51-60																	
<50																	
131							3										
121-130							2										
111-120																	
101-110							1										
91-100																	
81-90																	
71-80																	
61-70																	
51-60																	
41-50							1										
31-40							3										
30																	
Alert																	
Confusion																	
Voice																	
Pain																	
Unresponsive																	
>39.1°							2										
38.1-39.0°							1										
37.1-38.0°																	
36.1-37.0°																	
35.1-36.0°							1										
00							3										
NEWS Total																	
Monitoring frequency																	
Escalation of care (Y/N)																	
Screened for sepsis? (Y/N)																	
Sepsis pathway started (Y/N)																	
Initials																	
Coma Score (See back)																	
TRACOL and 2.2 Essential Adult Inpatient Observations Reporting and Escalation Policy																	
Pain Score																	

5 If no RED or 1/11 (flags present, treat according to standard NEWS-2 trigger thresholds. It is unlikely that the patient has sepsis at this stage, but keep an open mind and continue to screen for sepsis at every review.